

Patient Education & Experience Frameworks

A comprehensive reference across 26 frameworks, rubrics, and models — organized by function so you can stack the right tools for any project.

■ Clarity & Content

■ Process & Interaction

■ Behavior & Psychology

■ Outcomes & Experience

✓ Scored

⚠ Partial

✗ No score

Framework	Zone	Scoring	Metric / Score	Benchmark	Best Used For
① Clarity & Content Quality — Your Editors					
CCI CDC Clear Communication Index	Clarity	✓ Yes	0–100% (20 Yes/No items)	≥90% to pass	Final check before releasing any patient-facing material
PEMAT Patient Education Materials Assessment Tool (AHRQ)	Clarity	✓ Yes	Two separate % scores: understandability + actionability	>70% in both categories	Retroactive audits of existing materials; print and video
SAM Suitability Assessment of Materials	Clarity	✓ Yes	0–2 pts across 22 subcriteria → % score	70–100% Superior / 40–69% Adequate / <40% Not Suitable	Deep material audits across content, graphics, literacy demand, layout
DISCERN DISCERN Quality Instrument	Clarity	✓ Yes	16 questions, 1–5 scale each	Higher = better; context-dependent	Evaluating reliability and completeness of consumer health info
EQIP Ensuring Quality Information for Patients	Clarity	✓ Yes	36-item checklist with total score	Higher = better; no universal threshold	Thorough pre-publication audits of patient information documents
Readability Flesch-Kincaid / SMOG / Flesch Reading Ease	Clarity	✓ Yes	Grade level or 0–100 ease score (algorithmic)	Grade 6–8 target for general health materials	Quick automated check; pairs with CCI or PEMAT
Plain Language U.S. Federal Plain Language Guidelines	Clarity	✗ No	Qualitative heuristic checklist	Editorial judgment; no threshold	Writing and editing content at the drafting stage
NCI Pink Book National Cancer Institute Pink Book	Clarity	✗ No	Qualitative evaluation guidance	Pre/post testing recommended	Campaign planning and health communication strategy
② Process & Interaction Quality — Your Behavioral Referees					
OPTION Scale Option Scale for SDM	Process	✓ Yes	12-item scale, scored 0–4 each → total score	Higher = more patient involvement; research benchmarks vary	Auditing clinical interactions and shared decision-making practice
IPDAS International Patient Decision Aid Standards	Process	⚠ Semi	Qualifying + certifying criteria checklist	Certification threshold for qualifying criteria	Designing and auditing shared decision-making tools
AHRQ Toolkit Health Literacy Universal Precautions Toolkit	Process	⚠ Partial	Checklist completion; qualitative	Implementation-based; no universal score	Operational improvements in clinics; staff training
Ottawa DMSF Ottawa Decision Support Framework	Process	✗ No	Paired with Decisional Conflict Scale	Reduction in decisional conflict	Patient journey and decision aid design
SHARE SHARE Approach (AHRQ)	Process	✗ No	5-step model; qualitative	Implementation-based	Training clinicians in SDM; designing SDM workflows

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Teach-Back Teach-Back / Ask Me 3	Process	✗ No	Qualitative — can patient explain it back?	Patient demonstrates understanding	Design constraint for materials; clinician training
③ BEHAVIOR & PSYCHOLOGY — YOUR ENGINES					
COM-B COM-B Behaviour System	Behavior	⚠ Indirect	Behaviour Change Wheel companion tools	Intervention design guided by COM-B diagnosis	Product/service design; intervention mapping
HBM Health Belief Model	Behavior	✗ No	Qualitative constructs; measurable via surveys	Behavior adoption as outcome	Designing messaging strategy around chronic disease
TTM Transtheoretical Model (Stages of Change)	Behavior	✗ No	Stage classification (pre-contemplation → maintenance)	Stage progression over time	Tailoring interventions to patient readiness
EPPM Extended Parallel Process Model	Behavior	✗ No	Perceived threat vs. efficacy balance	Behavior change without paralysis or denial	Designing messaging around medication adherence and risk
WHO Risk Comms WHO Risk Communication Framework	Behavior	✗ No	Qualitative principles	Trust + behavior change as outcomes	Public health messaging; crisis communication
④ OUTCOMES & EXPERIENCE — YOUR SCOREBOARDS					
PAM Patient Activation Measure	Outcomes	✓ Yes	0–100 score → 4 activation levels	3–6 point increase over 6–12 months	Measuring behavior change readiness pre/post intervention
CAHPS CAHPS / HCAHPS Survey (AHRQ)	Outcomes	✓ Yes	Top-Box % (highest rating responses)	National average comparison; year-over-year	Benchmarking hospitals and clinics at scale
NPS Net Promoter Score	Outcomes	✓ Yes	-100 to +100	>50 is excellent; varies by sector	Executive-level CX tracking; digital product experience
Picker Picker Principles of Patient-Centered Care	Outcomes	✗ No	Paired with Picker survey instruments	Survey response benchmarks	Care model design; org-level patient experience strategy
Beryl PX The Beryl Institute PX Framework	Outcomes	✗ No	Qualitative organizational framework	Implementation + culture change	Org-level patient experience strategy
EBCD Experience-Based Co-Design	Outcomes	✗ No	Qualitative; co-design outputs	Patient + staff identified improvement areas	Participatory UX/design research in clinical environments
SEIPS Systems Engineering for Patient Safety	Outcomes	✗ No	Work system analysis outputs	Reduction in adverse events; improved workflow	Framing design decisions in clinical and systems contexts
Planetree Planetree Model	Outcomes	✗ No	Designation criteria; qualitative	Planetree designation	Org-level culture and care environment design

● RECOMMENDED STACKS

Auditing a Past Material

PEMATSAMCCI

Score retroactively across all three. Gaps between understandability and actionability tell the real story.

Designing a Decision Aid

OttawaDMSFIPDASOPTION

Framework → standards check → interaction audit. Covers design through delivery.

Behavior Change Campaign

COM-BTTMHBMCCI

Diagnose behavior, segment by readiness, then pressure-test your message clarity.

Proving Impact Post-Launch

PAMCAHPSNPS

Layer activation, experience, and satisfaction for a multi-dimensional outcome picture.

